

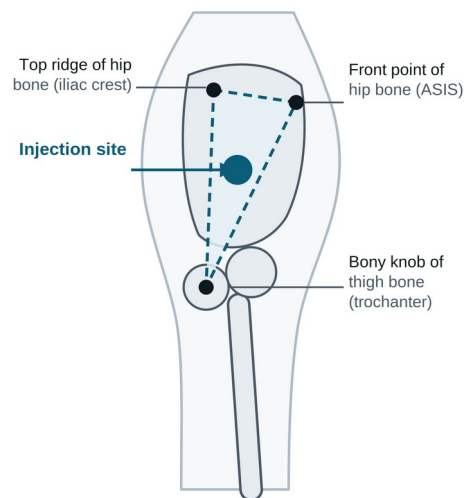
Testosterone undecanoate 1000 mg/4 mL • treatment-room card

Ventrogluteal IM injection • observe • act. Print and laminate. Version 2.0, 2 October 2026. Full SOP: <https://gamsa.au/medicines/testosterone-undecanoate>



Where the long-acting injection goes

The ventrogluteal site — the side of the hip



- 1 Palm flat on the bony knob, fingers pointing up
 - 2 Index finger on the front point of the hip bone
 - 3 Spread your middle finger back along the top ridge
- Needle into the V between them, straight in at 90°

1 Screen

Identity, consent, allergies, prior reaction to oily injection [1].

Defer or refer if: chest infection or breathless; previous POME; bleeding disorder or anticoagulant [1][11].

Product, 1000 mg/4 mL, batch, expiry.

2 Equipment

Draw up **18G**. Inject **21G × 38 mm**; **50 mm** if BMI > 30 [5][10].

Oxygen, adrenaline, monitoring, phone for 000 in the room [8].

3 Landmark and inject

Side-lying, upper hip relaxed. Heel of hand on greater trochanter, index finger to ASIS, middle finger along iliac crest: inject in the **V** [4][5].

90°, deep into muscle. **Aspirate**; blood → withdraw, new needle, new site [5][10].

Full 4 mL **very slowly**, about **2 minutes** [1][10].

Gentle pressure, dressing. **No massage** [5].

4 Observe

During, immediately after, and **at least 15 minutes** [1][3].

Record: product, batch, expiry, side, needle, time taken, clinician, outcome, next due (10–14 weeks; first interval ≥ 6 weeks) [1][3].

Book the next appointment before the patient leaves.

POME: cough, urge to cough, breathless, chest tight, sweating, dizzy, tingling, faint — within minutes

Stop. Sit up, legs out. **Oxygen**. Monitor pulse, BP, SpO₂, RR [7].

000 if severe, SpO₂ falling, or not settling within minutes [7].

Usually settles within an hour with support [6][7].

ANAPHYLAXIS: rash, swelling, wheeze, stridor, hypotension, collapse

Lie flat, legs raised; sit with legs out only if breathing is the problem. Do not stand [8].

Adrenaline 1:1000 IM, outer mid-thigh: 0.01 mg/kg, max 0.5 mg (0.5 mL). Call **000**. Repeat after 5 min if no better [8].

Oxygen. Hospital observation ≥ 4 h after last dose [8].

Afterwards

Notify prescriber. Adverse event form. Report to TGA: aems.tga.gov.au [9].